

Centre des opérations d'urgences de sante publique « COUSP-RDC »

Situation Report of the 17th Ebola Virus Disease Outbreak / DRC SitRep N° 009/MVB_24/2026

Provinces affected (3)	ITURI, NORTH KIVU, SOUTH KIVU
Affected Health Zones (11)	ITURI : Aru, Bunia, Kilo, Mongbwalu, Nizi, Nyankunde and Rwampara NORTH KIVU : Butembo, Goma and Katwa SOUTH KIVU : Miti-Murhesa
Reporting date	May 23, 2026
Publication date	May 24, 2026

							
101	10	904	119	ND	ND	0	ND
Total confirmed cases	Total deaths among confirmed cases	Cumulative cases suspects	Total Deaths suspects	Occupancy rate among suspects	Occupancy rate among experienced staff	Healed	Cumulative confirmed cases of CTE

Table I. Distribution of confirmed cases and deaths by health zone in the affected provinces as of 23/05/2026

Affected provinces	Health zones with case confirmed	New case confirmed	New deaths among confirmed cases	New case probable	New case suspects	New contacts listed	Cured of day
Ituri	Bunia	0	0	ND	22	43	0
	Rwampara	0	0	ND	3	0	0
	Mongbwalu	9	0	ND	ND	0	0
	Bamboo	0	0	ND	ND	ND	0
	Nyankunde	0	0	ND	ND	0	0
	Nizi	0	0	ND	0	7	0
	Kilo	0	0	ND	8	ND	0
Subtotal		9	0	0	33	50	0
North Kivu	Goma	0	0	ND	ND	ND	0
	Butembo	0	0	ND	ND	ND	0
	Katwa	1	0	ND	ND	ND	0
South Kivu	Miti Murhesa	0	0	ND	ND	ND	0
Subtotal		0	0	0	0	0	0
Total		10	0	0	33	50	0



A tripartite cross-border collaboration meeting between the Democratic Republic of Congo, the Republic of Uganda, and the Republic of South Sudan was held in Kampala on May 23, 2026, to strengthen preparedness and response to the 17th Ebola Virus Disease (Bundibugyo strain) outbreak. This activity aimed to harmonize surveillance mechanisms, alert notification, suspected case management, and cross-border contact tracing at Points of Entry (PoE) and Checkpoints.

I. CONTEXT

Ituri Province is located in the northeast of the Democratic Republic of Congo and shares a long border with Uganda and South Sudan. For more than two decades, it has faced a chronic humanitarian crisis linked to armed conflict and recurring population displacements. It has an estimated population of over 8 million, including more than one million internally displaced persons.

In September 2018, this province was affected by an Ebola virus disease epidemic that was raging in the neighbouring province of North Kivu.

The Mongbwalu health zone is one of the 36 health zones in the Ituri province.

Considered the starting point of the epidemic, it is located in the Djugu territory, where several armed groups operate. It is situated 70 km from Bunia, the capital of Ituri province, with frequent population movements towards Uganda. This locality is characterized by several construction sites that attract people from other territories within the province and from neighboring provinces.

II. HISTORY OF THE EPIDEMIC

On May 5, 2026, social media reported a surge in deaths of unknown cause, with nearly 50 deaths already recorded in the Mongbwalu health zone. Following confirmation of this alert, the Chief Medical Officer of the Mongbwalu health zone shared the preliminary investigation report on May 9, 2026. On May 11, 2026, provincial health authorities held an emergency meeting and decided to deploy rapid response teams to Mongbwalu and Rwampara, which had reported suspected deaths from viral hemorrhagic fever among patients hospitalized at the Rwampara Evangelical Medical Center, who had been transferred from Mongbwalu.

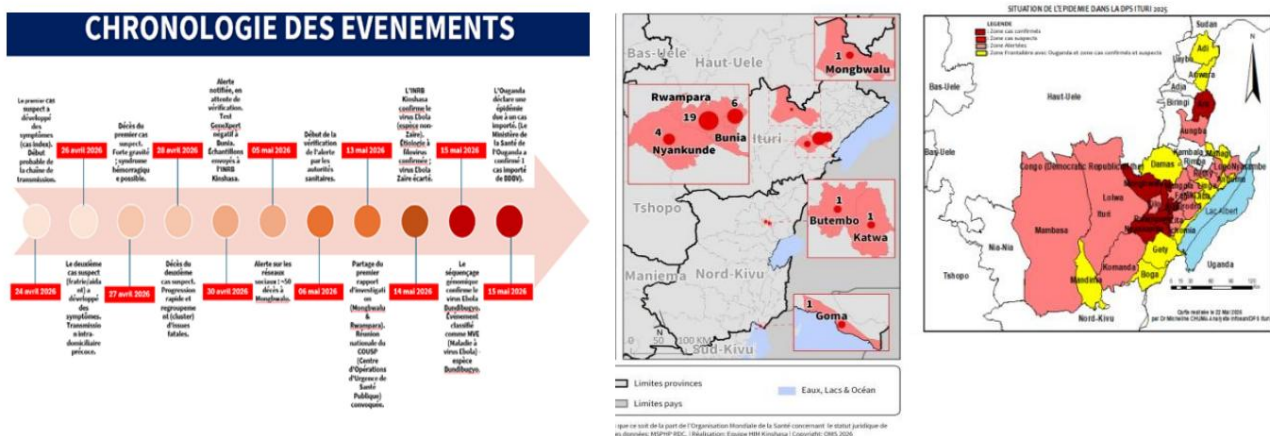
Investigations conducted on May 12, 2026, simultaneously in the Mongbwalu and Rwampara health zones by the Rapid Intervention Teams of the Ituri Provincial Health Division (DPS), revealed that most suspected patients or deaths presented with fever, headache, vomiting, and severe physical asthenia. Deaths occurred 2 to 3 days after the onset of illness, with symptoms including hematemesis, epistaxis, or blackish vomit. During the investigations in the Rwampara health zone, 20 samples were collected from suspected cases or deaths. The majority of samples were taken from the Mongbwalu health zone. Preliminary tests conducted at the Bunia Laboratory were all negative for Ebola Zaire, dengue fever, rotavirus, cholera, malaria, *Yersinia pestis*, measles, and COVID-19. These samples were sent to the National Institute of Biomedical Research (INRB) in Kinshasa for further testing.

As of May 14, 2026, **13 of these 20 samples were tested, and 8 were found to be positive** for non-Zairean Ebola virus disease at the INRB in Kinshasa. On May 15, 2026, sequencing of the positive samples identified the BUNDIBUGYO strain, *allowing the Minister of Public Health, Hygiene and Social Welfare (MSHPS) to officially declare the 17th Ebola virus disease outbreak.*

Ebola virus disease (Bundibugyo strain), thus marking the 17th epidemic in the country's history

As of May 17, 2026, in consultation with member states, the WHO Director-General considers that the event taking place in DRC and Uganda meets the criteria of the definition set out in Article 1 of the IHR, and declared that this constitutes a Public Health Emergency of International Concern while specifying that the risk at the global level is low but high at the regional level.

On May 18, 2026, the Executive Director of Africa CDC officially declared a Continental Public Health Emergency (CPHE) under Article 3 of its statutes. This declaration followed an emergency meeting of the Africa CDC Advisory Group, which determined that the worsening outbreak posed an acute risk to continental health security. As a result, the Continental Incident Management Support Team was activated to provide support to affected and at-risk Member States in their response and preparedness efforts.



III. EPIDEMIOLOGICAL SITUATION UPDATE

- 10 New Confirmed Cases (Mongbwalu: 9 and Katwa: 1)
 - Cumulative number of confirmed cases in Ituri as of May 23, 2026: 94 cases, North Kivu 6 cases and South Kivu 1 case
 - Three provinces affected: ITURI, NORTH KIVU and SOUTH KIVU: •
- ITURI: epicenter with 7/36 health zones affected (Rwampara, Bunia, Mongbwalu, Nyankunde, Nizi, Kilo and Aru.)
- NORTH KIVU: 3/35 health zones affected (Goma, Butembo and Katwa)
 - SOUTH KIVU: 1/34 ZS hit (Miti-Murhesa)

IV. RESPONSE ACTIONS

COORDINATION

- Removal of EDS materials from the PAM depot;

EPIDEMIOLOGICAL SURVEILLANCE

- Briefing on alert tracking at the DPS level including members of the civil protection (2 people);
- Carrying out field surveillance activities (*reporting alerts, investigations, active search, listing and follow-up of contacts, retrospective documentary review of cases and deaths, etc.*).

Table II. Summary of the alert status as of May 23, 2026

Health Zones Report	alerts (21/05)	New alerts (22/05)	Total alerts for today (22/05)	Alerts cases	Alerts deceased	Alerts investigated	No alerts investigated	Alerts validated	Alerts invalidated	Case suspects samples taken	Case suspects No samples taken	Case suspects isolated	Live suspects No isolated
BUNIA	6	21	27	21	6	25	2	22	3	5	17	22	0
RWAMPARA	7	9	16	10	6	10	6	7	3	7	0	3	4 (death)
NYANKUNDE	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND
MONGBWAL	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND
NIZI	0	1	1	0	1	1	0	1	0	1	0	0	1 (death)
ARU	0	1	1	1	0	1	0	1	0	1	0	1	0
KILO	0	18	18	18	0	8	10	8	0	8	0	8	0
BAMBU	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND
ADI	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND
LITA	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND
FATAKI	0	2	2	2	0	2	0	2	0	2	0	2	0
LOGO	0	4	4	2	2	4	0	4	0	4	0	2	2 (death)
AUNGBA	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND
RETHY	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND
KOMANDA	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND
NYARAMBE	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND
TCHOMIA	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND
DRODRO	0	2	2	2	0	2	0	2	0	2	0	2	0
MAMBASSA	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND
MANGALA	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND
LOLWA	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND	ND
TOTAL	13	58	71	56	15	53	18	47	6	30	17	40	7

Table III. Summary of contact identification and follow-up as of May 23, 2026

Areas of health	Daily monitoring						tracking	Total	Reason for not being seen						the
	Postponed from	May New	day Contact	Contacts	Releases	They			Simple	Lost	hours) Never	Refusal	Displaced	pass Reco	
BUNIA	361	43	404	71	0	0	17.6 %	333	0	0	0	0	0	333	404
RWAMPARA	621	0	621	ND 0 ND	ND ND ND	ND ND ND	ND ND ND	ND ND ND							621
NYANKUNDE	365	0	365 ND	0 ND	ND ND ND	ND ND ND	ND ND ND	ND ND ND							365
MONGBWALU	222	0	222	ND 0 ND	ND ND ND	ND ND ND	ND ND ND	ND ND ND							222
NIZI	15	7	22	15	0	1	68.2 %	7	0	0	0	0	0	7	22
ARU	ND ND ND			ND ND		ND ND ND	ND ND ND	ND ND ND							0
KILO	ND ND ND			ND ND		ND ND ND	ND ND ND	ND ND ND							0
Total	1603	142	1745	351	0	13	1394	99		1	0	0	0	1294	1745

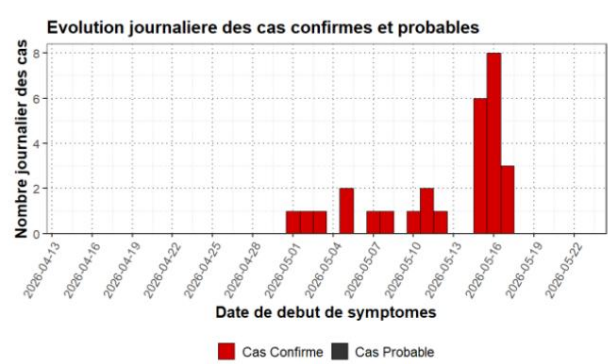
Table IV. Distribution of suspected and confirmed Ebola virus disease cases, deaths in the health zones of the Ituri, North Kivu and South Kivu Provincial Health Directorates as of May 23, 2026*

Provinces	Health zones	Number of suspected cases	Number of suspicious deaths**	Number of cases Confirmed	Number of contacts
Ituri	Aru	4	1	1	0
	Bamboo	6	2	0	183
	Bunia	249	48	24	404
	Kilo	6	2	1	0
	Mongbwalu	339	88	19	222
	Nizi	6	2	2	22
	Nyankunde	45	8	9	365
	Rwampara	228	69	32	621
	Samples without a datasheet	0	0	6	0
North Kivu	Butembo	7	0	1	ND
	Goma	0	0	1	ND
	Katwa	13	0	4	ND
South Kivu	Miti Murhesa	1	0	1	ND
Total		904	119	101	1817

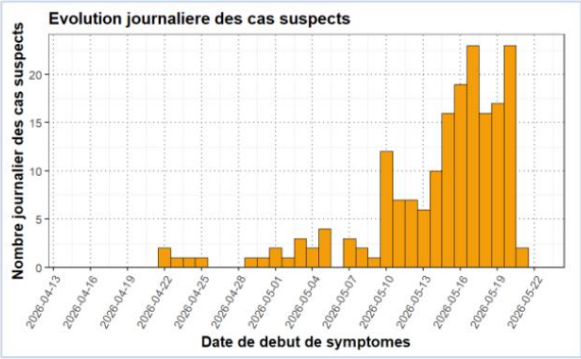
*This data is subject to change after harmonization of data at different levels of the health pyramid with the incident management system.

** The classification will be confirmed after further investigation.

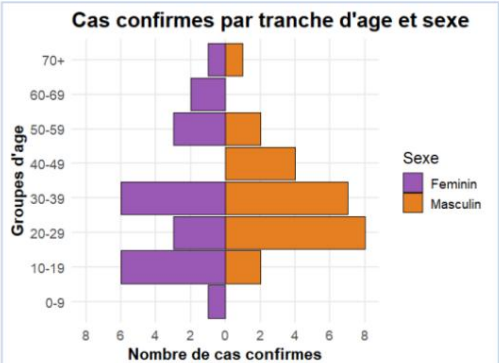
Figure 1. Evolution of confirmed cases in the Ituri DPS



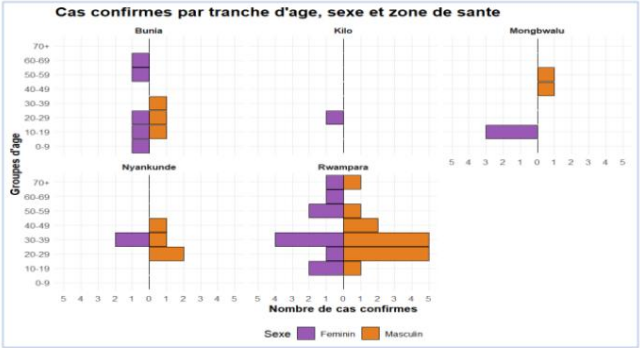
Epidemic curve of confirmed and probable cases of Ebola virus disease (Ituri Province)
Source: DHIS2 Tracker as of May 22, 2026 (n=48)
suspected cases: 28 with date of symptom onset



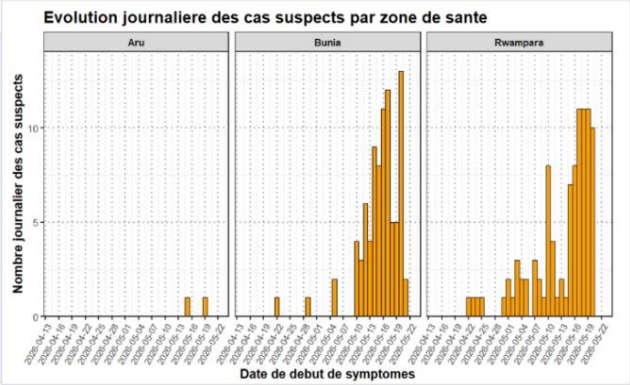
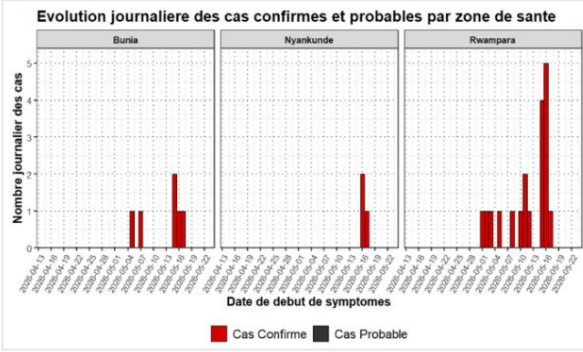
Epidemic curve of suspected cases of Ebola virus disease (Ituri Province)
Source: DHIS2 Tracker as of May 22, 2026 (n=311)
suspected cases: 232 with date of symptom onset



by date of symptom onset (source: DHIS2 Partial Tracker)



Distribution by Sex and Age of confirmed EVD cases as of May 22, 2026



Point of Entry and Point of Control (PoE/PoC) Surveillance

• Table V. Data Summary (PoE/PoC) of 23/05/2026

Health Zone	PoE/PoC	Travelers	Traveler screened	Total hand washing	Aware travelers	All alerts	Total refusal Handwashing	Total refusal screening
NIZI	NIZI BRIDGE	ND	ND	ND	ND	ND	ND	ND
BUNIA	AIRPORT	1332	1332	1332	1332	0		0
	MUDZIBALA	2028	1924	1924	2028	0	0 104	104
	BRIDGE SIMBILIABO	ND	ND	ND	ND	ND	ND	ND
RWAMPARA SHARI	BRIDGE	1800	1739	1739	1800	0	61	61
	DELE	1109	1057	52	1109	0	52	52
	CHAI	243	200	200	243	0	43	43
TCHOMIA	TCHOMIA					0		0
	NYAMUSASI	0 167	0 148	0 148	0 167	0	0 19	19
	KASENYI	ND	ND	ND	ND	ND	ND	ND
	MANDJE	ND	ND	ND	ND	ND	ND	ND
NYAKUNDE MARABO		ND	ND	ND	ND	ND	ND	ND
	NYAK ENTRANCE.	ND	ND	ND	ND	ND	ND	ND
	SOTA	ND	ND	ND	ND	ND	ND	ND
	TALOLO BRIDGE	ND	ND	ND	ND	ND	ND	ND
MONGBWALU POIPO		ND	ND	ND	ND	ND	ND	ND
	MAHIDEDE	ND	ND	ND	ND	ND	ND	ND
	PLITO BRIDGE	ND	ND	ND	ND	ND	ND	ND
	KIRIKOU BRIDGE	ND	ND	ND	ND	ND	ND	ND
ARU	OFOO	392	358	358	392	0	34	34
	ODHUMONI	317	289	289	317	0	28	28
	OMBAYI					0	23	23
	NO COMPLAINT	175,121	152,103	152,103	175,121	0	18	18
	VIS À VIS	87	71	71	87	0	16	16
	KENGEZI-BASE	51	37	37	51	0	14	14

LABORATORY

Total	
Total samples received: 418	
Re-sampling (control) 3	
Total samples analyzed	211
Cumulative positives	101

DISTRIBUTION OF SAMPLES TESTED AT THE BUNIA PAR NATURE LAB

Date	Origin	shadows of the swatches	decanting decorations		Retaliation
			ang	ab	
	united				
	ambu mine				
	oga				
	a unde				
	R ampara ita				
	ogo				
	ongb alu				
O					

DISTRIBUTION OF SAMPLES TAKEN

			Diagnosis				Follow up	
Date	Origin/ ZS	Reprevele ment	Results P1		Results P2		Results P3 to P5	
			Positive	Negative	Positives	Negatives	Positive	Negative
22/05/2026	Nyakunde	0	0	0	0	0	0	0
	Mongbwalu	0	0	0	0	0	0	0
TOTAL		0	0	0	0	0	0	0

DISTRIBUTION OF SAMPLES TESTED AT THE BUNIA PAR NATURE LAB

Date	Origin /ZS	Number of samples tested	Sample types		Represent veiling	Positives		Negatives		Undetermined	
			Sang Swab			Blood Swab	Blood Swab	Blood Swab			
23/05/ 2026	Nyakunde	3	3	0	0	0	0	3	0	0	0
	Mongbwalu	19	19	0	0	9	0	10	0	0	0
TOTAL		22	22	0	0	9	0	13	0	0	0

• Sending 257 samples to INRB Kinshasa (135 unanalyzed samples (123 blood and 12 swabs), 74 QC and 33 sequencing).

RISK COMMUNICATION AND COMMUNITY ENGAGEMENT (RCCE)

- Advocacy efforts with the mayors of the municipalities of Shari, Nyakasanza, and Mbunya to secure their commitment to supporting communication activities related to the Ebola response.
- Briefing of 19 department heads operating at Bunia airport, including 15 men and 4 women, on the Ebola virus disease and prevention measures in public areas
- Awareness-raising of 38 livestock farmers and wholesale traders at the Bunia industrial slaughterhouse on preventive measures against Ebola virus disease, including 16 women and 22 men.
- Educational talk with 30 women during a CPS session on preventive measures to MVE, ZS Bunia, AS BIGO

INFECTION PREVENTION AND CONTROL

ZS Bunia

- Joint PCI-PEC supervision of CT Bunia for circuit definition;
- Briefing of a service provider and 1 TS in the CSR LEMBABO on waste management;
- Initial scorecard assessment at SOFEPADI/AS AERO (38.8%) and TS briefing on handwashing technique.
- 5 EDS carried out out of 7 alerts (1 HGR Bunia, 2 SOFEPADI, 1 Clinique CITADELLE, 1 Clinique les Merits);
- Decontamination of the 4 rooms where the deceased cases were located (1 Bunia General Hospital, 2 SOFEPADI, 1 Clinique CITADEL);
- Briefing of the 2 midwives at the UNIBU hospital on waste management, the wearing and removal of PPE, and provision of 10 bins

ZS Rwampara

- Briefing of 14 service providers and 2 TS on the management of biomedical waste;
- 2 EDS carried out on 2 alert cases in the community (SHARI)
- Decontamination in 2 households around the case.

ZS Mongbwalu

- EDS carried out on 8 alert cases (1 Quartier Dépôt, 4 ZUBUDA, 2 SHUNI, 1 at Deux cent)
- Lack of mobility for teams, the number of daily death alerts > 10;
- The response team's services are insufficient to meet the need.
- Some families lack coffins, which delays the EDS (Emergency Response Scheme), hence the possibility of... make available

ZS NYAKUNDE

- Decontamination of a household in MARABO and interning IEM Nyakunde.

DPS North Kivu

- Decontamination of Matanda Hospital where the confirmed case from the Katwa health zone is staying;
- Matanda Hospital PCI scorecard evaluation: 70.6%; • Case mapping completed;
- Status of the CTE for the release of PCI input needs (PPE, water supply, waste management, patient circuit).

LOGISTICS

• Preparation for the shipment of IPC supplies and sampling kits for the Mongbwalu Health Zone • Supply of IPC supplies to the 8 Category 1 Health Establishments: • Bambu General Referral

Hospital (Djugu Health Zone) • Lolwa

General Referral Hospital

• HGR BUNIA

• HGR NIZI

Bunia Health Zone: Bunia City Hospital, Emerite Hospital, PNC Hospital, Unibu Clinic, Lambabo Health Center

• Reception of 5 vehicles, UG-PDSS support

• Logistical support for the interventions of the various pillars in the field, moving towards the axes:

Rwampara, Nyakunde, Bunia, Kilo and Mungbwalu

HOLISTIC CARE

- 9 positive cases today, all from Mongbwalu;
- One of 3 tents burned in Mongbwalu;
- 18 suspects escaped in Mongbwalu after yesterday's incident;
- A patient who escaped from Rwampara refuses to return despite being made aware of the situation;
- 1 Confirmed escapee PPL from Nyankunde readmitted in critical condition;
- 5 positive cases discharged from CH CME (Clinical Improvement) whose results came back positive;
- Patients wander through other rooms in the form of a revolt;
- 1 positive patient from CS Hoho released under pressure (Interview unsuccessful) ensured by our APS this morning);
- Nutritional PEC started with Alima;
-

- Equipment provided: Oxygen concentrator, multiparameter monitor.

Table VI. Patient movement in healthcare facilities as of [date]

23/05/2026

Indicators		ZS Bunia	ZS Rwampara	ZS Mongbwalu	ZS Nyankunde	ZS Aru	Total General
Report (Number of patients present in bed at the end of the previous day)		41	21	28	26	2	118
News Today's admissions	PPL	3	2	0	3	0	8
	Others	6	2	3	4	0	15
	Total new admissions for today	9	4	3	7	2	25

Total number of people in isolation		50	25	31	33	2	141
Cumulative number of patients admitted to isolation		112	54	65	36	3	270
Outings	Healed	0	0	0	0	0	0
	Deceased	7	4	1	0	0	12
	Total deaths	57	34	48	1	1	141
	No Case	0	0	0	0	0	0
	Escapees	5	0	18	0	0	23
	Transferred to HGR	0	2	0	0	0	2

SECURITY

- The security situation in the city of Bunia (Bunia and Rwampara health zones) is relatively calm
- Security briefing for deployed teams.

V. CHALLENGES

- Community resistance to the decontamination of their households (Rwampara Health Zone)
- Contact tracing remains suboptimal, requiring cascading briefings (IS, IT, community agents) at the level of active and high-risk health zones;
 - Difficulty in mobility for teams at the provincial level and in the ZS (one vehicle per ZS, no motorcycles) remains.
 - Insufficient Protective Kits for field teams;
- Non-deployment of the multidisciplinary EIR in the **Mongbwalu health zone** **ZS Aru** and ZS high risk.

VI. Recommendation

- Make equipment available for provincial and zonal alert centers (**Resp: Coordination**);
- Rapidly deploy a multidisciplinary team in the Mongbwalu health zone as well as in the Aru and Kilo health zones (**Resp: Coordination**);
- Organize cascading briefings for ZS stakeholders on MVE surveillance (**Resp: Coordination**).

PHOTOS



A tripartite cross-border collaboration meeting between the Democratic Republic of Congo, the Republic of Uganda and the Republic of South Sudan was held in Kampala on May 23, 2026, as part of strengthening preparedness and response to the 17th Ebola Virus Disease (Bundibugyo strain) outbreak.

**FOR ALL
INFORMATION
ADDITIONAL,
PLEASE CONTACT**

For the National Institute of Public Health (INSP) of the DRC

The Director General of INSP

Dr. MWAMBA KAZADI Dieudonné

E-mail: diedonne.mwamba@insp.cd Tel. :
+243 816 040 145

The COU-SP Coordinator

Professor NGANDU Christian

E-mail: Christian.ngandu@insp.cd
Tel. : +243998091915

The Incident Manager

Professor Pierre AKILIMALI

E-mail: pierre.akilimali@insp.cd
Tel. : +243815800288

For the World Health Organization (WHO/DRC)

The WHO Representative in the DRC

Dr Anne Marie ANCIA

Email: anciaa @who.int

The WHO EPR Cluster Lead Team in the DRC

Dr FOTSING Richard E-

mail: fotsingri@who.int

For the Africa CDC

Justus Nsio Mbata

Email: JustusNs@africacdc.org

